

# CJC-1295

Modified GRF 1-29 • GHRH Analog • Research Overview • July 2026

## WHAT IT IS

CJC-1295 is a synthetic 30-amino-acid analog of growth hormone-releasing hormone (GHRH). It is a tetrasubstituted version of the first 29 amino acids of native GHRH, engineered for resistance to DPP-4 enzymatic degradation. It binds GHRH receptors on pituitary somatotroph cells and stimulates endogenous GH release via the cAMP/PKA pathway. Unlike recombinant GH, it works through the body's own pituitary and preserves natural somatostatin feedback.

## TWO FORMS — CRITICAL DISTINCTION

| FEATURE           | WITH DAC                                               | WITHOUT DAC (Mod GRF 1-29)                     |
|-------------------|--------------------------------------------------------|------------------------------------------------|
| Half-life         | ~5.8–8.1 days (albumin-bound)                          | ~20–30 minutes                                 |
| GH Pattern        | Sustained elevation ("GH bleed") with preserved pulses | Sharp, physiological pulses                    |
| Typical Dose      | 1–2 mg SC once weekly (or split 2×/wk)                 | 100–200 mcg SC 1–3× daily (often + Ipamorelin) |
| Convenience       | High — infrequent injections                           | Lower — multiple daily pins                    |
| Physiological Fit | Lower (tonic stimulation)                              | Higher (mimics natural pulses)                 |
| Human Data        | Yes — Teichman et al. 2006 Phase 1                     | None published as controlled trials            |

## KEY HUMAN EVIDENCE (DAC FORM ONLY)

Teichman et al., J Clin Endocrinol Metab 2006 — two randomized, placebo-controlled, double-blind ascending-dose Phase 1 trials in healthy adults (21–61 yrs). Single SC injection produced dose-dependent mean GH increases of **2- to 10-fold** lasting ≥6 days and IGF-1 increases of **1.5- to 3-fold** lasting 9–11 days. Estimated half-life **5.8–8.1 days**. With multiple weekly doses, IGF-1 stayed elevated up to 28 days. No serious adverse reactions at 30–60 mcg/kg; described as relatively well tolerated. Pulsatile GH secretion was preserved. Development was later discontinued; a larger HIV-lipodystrophy trial was terminated after one subject death (investigators judged unrelated; data unpublished).

## REPORTED EFFECTS & POTENTIAL BENEFITS

- **Documented:** Reliable, dose-dependent elevation of endogenous GH and IGF-1.
- **Commonly reported (clinic/community):** Improved deep sleep quality & recovery, mild body-composition support (lean mass / fat), better skin quality, joint comfort.
- **Theoretical / extrapolated:** Recovery, collagen synthesis, metabolic support — none of these endpoints have been proven in controlled CJC-1295 efficacy trials.

## COMMON RESEARCH / COMPOUNDING PROTOCOLS

**With DAC:** 1–2 mg SC once weekly (or 500–1000 mcg twice weekly). Typical cycle 8–12 weeks on / 4 weeks off.  
**Without DAC (Mod GRF):** 100–200 mcg SC 1–3× daily, frequently stacked with Ipamorelin 200–300 mcg in the same syringe. Best timed fasted (especially bedtime for nocturnal GH synergy). 5-on/2-off or continuous short cycles common.  
**Administration:** Subcutaneous (abdomen, flanks, thighs). Reconstitute lyophilized powder with bacteriostatic water; refrigerate and use within ~28 days.

## SAFETY PROFILE & CONSIDERATIONS

Published human safety data are limited to short Phase 1 exposure. **Common mild effects:** injection-site reactions (redness/irritation — frequent), transient flushing/warmth ("CJC flush"), mild water retention, occasional headache or tingling. **Mechanistic concerns** with sustained GH/IGF-1 elevation: potential insulin resistance, fluid retention, and theoretical long-term proliferative risk (elevated IGF-1 linked epidemiologically to certain cancers). FDA has flagged CJC-1295 for compounding concerns (immunogenicity, impurities, reported increased heart rate / vasodilatory reactions). **Not FDA-approved** for any indication. Generally avoided with active malignancy or cancer history. Monitor IGF-1, glucose/insulin, and clinical symptoms under medical supervision.

## RELATION TO TESAMORELIN & OTHER GHRH ANALOGS

Tesamorelin is the only FDA-approved GHRH analog (indicated for HIV-associated lipodystrophy) with a ~2-hour half-life and robust clinical data on visceral fat reduction. CJC-1295 (especially the no-DAC form) is pharmacologically closer to sermorelin / modified GRF but lacks the same regulatory and efficacy evidence base. Many prefer no-DAC + Ipamorelin for more physiological pulsing versus the sustained profile of the DAC version.

DISCLAIMER: For educational and research reference only. CJC-1295 is not approved by the FDA for human therapeutic use and is sold as a research chemical. Use outside controlled research settings carries unknown risks. Consult a qualified physician before considering any peptide protocol. Information synthesized from published literature (primarily Teichman 2006) and public peptide research summaries as of July 2026.